DIAGNOSTIC REPORT

agilus diagnostics

PATIENT NAME : SHAHAZEER UMMER D		**REF. DOCTOR :** DR. ROHITH PINTO
SHAHAZEER UMMER D	ACCESSION NO : **0298ZA001494** PATIENT ID : SHAHM050602298 CLIENT PATIENT ID: ABHA NO :	AGE/SEX : 23 Years Male DRAWN : 06/01/2026 08:03:02 RECEIVED : 06/01/2026 08:02:24 REPORTED : 06/01/2026 11:10:31

Test Report Status **Final**	Results	Biological Reference Interval	Units

ELECTROCARDIOGRAM

ECG	TESTED		

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SCREENING ECHOCARDIOGRAM

CLINICAL PROFILE

TESTED

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HAEMATOLOGY			
GLYCOSYLATED HEMOGLOBIN(HBA1C),EDTA WHOLE BLOOD			
HBA1C	5.3	Non-diabetic: < 5.7 Pre-diabetics: 5.7 - 6.4 Diabetics: > or = 6.5 Therapeutic goals: < 7.0 Action suggested : > 8.0 (ADA Guideline 2021)	%
METHOD : HPLC			
ESTIMATED AVERAGE GLUCOSE(EAG)	105.4	< 116.0	mg/dL

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BIOCHEMISTRY			
GLUCOSE FASTING (FBS), FLUORIDE PLASMA			
FBS-FASTING BLOOD SUGAR(GLUCOSE)	**103 High**	(Normal <100,Impaired fasting glucose:100 to 125,Diabetes mellitus:>=126(on more than 1 occasion)(ADA guidelines 2024)	mg/dL
CREATININE, SERUM			
CREATININE	0.93	0.90 - 1.30	mg/dL
SODIUM, SERUM			
SODIUM, SERUM METHOD : ION SELECTIVE ELECTRODE TECHNOLOGY	142	137 - 145	mmol/L
POTASSIUM, SERUM			
POTASSIUM, SERUM METHOD : ION SELECTIVE ELECTRODE TECHNOLOGY	4.5	3.6 - 5.0	mmol/L
URIC ACID, SERUM			
URIC ACID	**8.7 High**	3.5 - 7.2	mg/dL

Interpretation(s)
GLUCOSE FASTING (FBS), FLUORIDE PLASMA-**TEST DESCRIPTION**
Normally, the glucose concentration in extracellular fluid is closely regulated so that a source of energy is readily available to tissues and sothat no glucose is excreted in the urine.
Increased in:Diabetes mellitus, Cushing' s syndrome (10 – 15%), chronic pancreatitis (30%). Drugs:corticosteroids,phenytoin, estrogen, thiazides.
Decreased in :Pancreatic islet cell disease with increased insulin,insulinoma,adrenocortical insufficiency,hypopituitarism,diffuse liver disease,

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malignancy(adrenocortical,stomach,fibrosarcoma),infant of a diabetic mother,enzyme deficiency diseases(e.g.galactosemia),Drugs-insulin,ethanol,propranolol sulfonylureas,tolbutamide,and other oral hypoglycemic agents.
NOTE: While random serum glucose levels correlate with home glucose monitoring results (weekly mean capillary glucose values),there is wide fluctuation within individuals.Thus, glycosylated hemoglobin(HbA1c) levels are favored to monitor glycemic control.
High fasting glucose level in comparison to post prandial glucose level may be seen due to effect of Oral Hypoglycaemics & Insulin treatment,Renal Glyosuria,Glycaemic index & response to food consumed,Alimentary Hypoglycemia,Increased insulin response & sensitivity etc.

Glucose fasting, plasma as per ADA Guideline
NORMAL RANGE: <100 mg/dl
PRE-DIABETIC RANGE: 100-125 mg/dl (Impaired Fasting Glucose)
DIABETIC RANGE: = 126 mg/dl
CREATININE, SERUM-**Higher than normal level may be due to:**
• Blockage in the urinary tract, Kidney problems, such as kidney damage or failure, infection, or reduced blood flow, Loss of body fluid (dehydration), Muscle problems, such as breakdown of muscle fibers, Problems during pregnancy, such as seizures (eclampsia)), or high blood pressure caused by pregnancy (preeclampsia)
Lower than normal level may be due to:• Myasthenia Gravis, Muscuophy
SODIUM, SERUM-Increased in dehydration, cushings syndrome, aldosteronism Decreased in Addisons disease, hypopituitarism,liver disease.
POTASSIUM, SERUM-Hypokalemia (low K) is common in vomiting, diarrhea, alcoholism, folic acid deficiency and primary aldosteronism. Hyperkalemia may be seen in end-stage renal failure, hemolysis, trauma, Addisons disease, metabolic acidosis, acute starvation, dehydration, and with rapid K infusion.
URIC ACID, SERUM-**Causes of Increased levels:**-Dietary(High Protein Intake,Prolonged Fasting,Rapid weight loss),Gout,Lesch nyhan syndrome,Type 2 DM,Metabolic syndrome **Causes of decreased levels**-Low Zinc intake,OCP,Multiple Sclerosis

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BIOCHEMISTRY - LIPID			
LIPID PROFILE, SERUM			
CHOLESTEROL, TOTAL	140	Desirable: <200 BorderlineHigh : 200-239 High : > or = 240	mg/dL
TRIGLYCERIDES	84	Desirable: < 150 Borderline High: 150 - 199 High: 200 - 499 Very High : > or = 500	mg/dL
HDL CHOLESTEROL	**27 Low**	< 40 Low > or = 60 High	mg/dL
LDL CHOLESTEROL, DIRECT	98	Adult levels: Optimal < 100 Near optimal/above optimal: 100-129 Borderline high : 130-159 High : 160-189 Very high : = 190	mg/dL
NON HDL CHOLESTEROL	113	Desirable: Less than 130 Above Desirable: 130 - 159 Borderline High: 160 - 189 High: 190 - 219 Very high: > or = 220	mg/dL
VERY LOW DENSITY LIPOPROTEIN	16.8	< or = 30.0	mg/dL
CHOL/HDL RATIO	**5.2 High**	3.30 - 4.40	
LDL/HDL RATIO	**3.6 High**	0.5 - 3.0	

Interpretation(s)

Serum lipid profile is measured for cardiovascular risk prediction. Lipid Association of India recommends LDL-C as primary target and Non HDL-C as co-primary treatment target.

Risk Stratification for ASCVD (Atherosclerotic cardiovascular disease) by Lipid Association of India

Risk Category	
Extreme risk group	A.CAD with > 1 feature of high risk group
	B. CAD with > 1 feature of Very high risk group or recurrent ACS (within 1 year) despite LDL-C < or = 50 mg/dl or polyvascular disease

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Very High Risk	1. Established ASCVD 2. Diabetes with 2 major risk factors or evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors. 2. Diabetes with 1 major risk factor or no evidence of end organ damage. 3. CKD stage 3B or 4. 4. LDL >190 mg/dl 5. Extreme of a single risk factor. 6. Coronary Artery Calcium - CAC >300 AU. 7. Lipoprotein a >/= 50mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors

Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age > or = 45 years in males and > or = 55 years in females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by LAI in 2020.

Risk Group	**Treatment Goals**		**Consider Drug Therapy**	
	LDL-C (mg/dl)	**Non-HDL (mg/dl)**	**LDL-C (mg/dl)**	**Non-HDL (mg/dl)**
Extreme Risk Group Category A	<50 (Optional goal < OR = 30)	< 80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	<OR = 30	<OR = 60	> 30	>60
Very High Risk	<50	<80	>OR= 50	>OR= 80
High Risk	<70	<100	>OR= 70	>OR= 100
Moderate Risk	<100	<130	>OR= 100	>OR= 130
Low Risk	<100	<130	>OR= 130*	>OR= 160

*After an adequate non-pharmacological intervention for at least 3 months.

References: Management of Dyslipidaemia for the Prevention of Stroke: Clinical Practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.

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CLINICAL PATH - URINALYSIS

URINE ROUTINE /URINE ANALYSIS

PHYSICAL EXAMINATION, URINE

COLOR	YELLOWISH
APPEARANCE	CLEAR

CHEMICAL EXAMINATION, URINE

PH	5.0	4.7 - 7.5
SPECIFIC GRAVITY	1.025	1.003 - 1.035
PROTEIN	NOT DETECTED	NOT DETECTED
GLUCOSE	NOT DETECTED	NEGATIVE
KETONES	NOT DETECTED	NOT DETECTED
BLOOD	NOT DETECTED	NOT DETECTED
BILIRUBIN	NOT DETECTED	NOT DETECTED
UROBILINOGEN	NORMAL	NORMAL
NITRITE	NOT DETECTED	NOT DETECTED
LEUKOCYTE ESTERASE	NOT DETECTED	NOT DETECTED

MICROSCOPIC EXAMINATION, URINE

RED BLOOD CELLS	NOT DETECTED	0 - 2	/HPF
PUS CELL (WBCS)	3-5	0-5	/HPF
EPITHELIAL CELLS	2-3	0-5	/HPF
CASTS	NOT DETECTED		
CRYSTALS	NOT DETECTED		
BACTERIA	NOT DETECTED	NOT DETECTED	
YEAST	NOT DETECTED	NOT DETECTED	

Interpretation(s)

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The following table describes the probable conditions, in which the analytes are present in urine

Presence of	Conditions
Proteins	Inflammation or immune illnesses
Pus (White Blood Cells)	Urinary tract infection, urinary tract or kidney stone, tumors or any kind of kidney impairment
Glucose	Diabetes or kidney disease
Ketones	Diabetic ketoacidosis (DKA), starvation or thirst
Urobilinogen	Liver disease such as hepatitis or cirrhosis
Blood	Renal or genital disorders/trauma
Bilirubin	Liver disease
Erythrocytes	Urological diseases (e.g. kidney and bladder cancer, urolithiasis), urinary tract infection and glomerular diseases
Leukocytes	Urinary tract infection, glomerulonephritis, interstitial nephritis either acute or chronic, polycystic kidney disease, urolithiasis, contamination by genital secretions
Epithelial cells	Urolithiasis, bladder carcinoma or hydronephrosis, ureteric stents or bladder catheters for prolonged periods of time
Granular Casts	Low intratubular pH, high urine osmolality and sodium concentration, interaction with Bence-Jones protein
Hyaline casts	Physical stress, fever, dehydration, acute congestive heart failure, renal diseases
Calcium oxalate	Metabolic stone disease, primary or secondary hyperoxaluria, intravenous infusion of large doses of vitamin C, the use of vasodilator naftidrofuryl oxalate or the gastrointestinal lipase inhibitor orlistat, ingestion of ethylene glycol or of star fruit (Averrhoa carambola) or its juice
Uric acid	arthritis
Bacteria	Urinary infectionwhen present in significant numbers & with pus cells.
Trichomonas vaginalis	Vaginitis, cervicitis or salpingitis

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SPECIALISED CHEMISTRY - HORMONE			
TSH 3RD GENERATION ULTRASENSITIVE, SERUM			
TSH (ULTRASENSITIVE)	0.973	0.270 - 4.200	µIU/mL
METHOD : ECLIA			

Interpretation(s)
TSH 3RD GENERATION ULTRASENSITIVE, SERUM-**TSH stands for thyroid stimulating hormone.**This hormone stimulates the Thyroid gland to make thyroid hormones that regulate the way our body uses energy. These also play an important role in regulating weight, temperature, muscle strength, and even your mood. TSH is made in a gland in the brain called the pituitary. When thyroid levels in our body are low, the pituitary gland makes more TSH. When thyroid levels are high, the pituitary gland makes less TSH. TSH levels that are too high or too low can indicate that thyroid is not working correctly.
There is a circadian rhythm of TSH secretion, with peak values at the onset of sleep and nadir concentrations during the afternoon hours. Peak and nadir concentrations differ by approximately +/- 50%. The effect on circulating T4 and T3 concentrations is not significant because of the large size of the extrathyroidal T4 pool.
In healthy subjects there is no significant impact of body weight, physical training, body habitus, posture, immobilization, mild to moderate exercise, or ambulatory status on thyroid function, and no significant geographic environmental variation. Nutrition also has a minimal impact except for variation in iodine intake. Subthreshold concentrations of iodine intake are associated with increased TSH secretion, goiter, increased thyroid iodine uptake, decreased T4 production, an increased T3/T4 secretion ratio, and an increased ratio of circulating T3/T4 concentrations. Excessive iodine intake can block thyroid hormone biosynthesis by inhibiting the enzymes involved in the biosynthetic process, resulting in reduced T4 secretion, increased TSH concentrations, goiter, and hypothyroidism if the iodine excess is chronic.
High TSH levels can mean your thyroid is not making enough thyroid hormones, a condition called hypothyroidism.**Low TSH levels** can mean your thyroid is making too much of the hormones, a condition called hyperthyroidism. A TSH test does not explain why TSH levels are too high or too low.
In cases of Subclinical hypothyroidism, a single test can be misleading, so a second test is usually done 2 or 3 months later. In both tests, the blood is taken at the same time of day because TSH levels can fluctuate over the course of 24 hours. Subclinical hypothyroidism is diagnosed when both TSH readings are high but the thyroid hormone thyroxine is still within the normal range.
Being severely overweight and certain medications can also increase TSH. TSH levels are likely to fluctuate more during pregnancy.
TSH values may be transiently altered because of Non thyroidal Illness like severe infections, liver disease, renal failure, heart failure, severe burns, trauma, surgery etc.
TSH levels that are slightly or only moderately elevated do not necessarily need to be treated. Some people who have high TSH levels never even develop symptoms.
It is also very common for TSH levels to return to normal in children and teenagers.
REF: 1. TIETZ Fundamentals of Clinical chemistry 2Guidlines of the American Thyroid association during pregnancy and Postpartum,2011.
TSH in pregnancy
There's reduction in both the lower and the upper limit of maternal TSH relative to the non-pregnant TSH reference range. This is because of elevated levels of serum hCG that directly stimulates the TSH receptor, thereby increasing thyroid hormone production. The largest decrease in serum TSH is observed during the first trimester. Thereafter, serum TSH and its reference range gradually increases in the second and third trimesters, but nonetheless remains lower than in non-pregnant women.

****End Of Report****
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1. It is presumed that the test sample belongs to the patient named or identified in the test requisition form.
2. All tests are performed and reported as per the turnaround time stated in the AGILUS Directory of Services.
3. Result delays could occur due to unforeseen circumstances such as non-availability of kits / equipment breakdown / natural calamities / technical downtime or any other unforeseen event.
4. A requested test might not be performed if:
   i. Specimen received is insufficient or inappropriate
   ii. Specimen quality is unsatisfactory
   iii. Incorrect specimen type
   iv. Discrepancy between identification on specimen container label and test requisition form
5. AGILUS Diagnostics confirms that all tests have been performed or assayed with highest quality standards, clinical safety & technical integrity.
6. Laboratory results should not be interpreted in isolation; it must be correlated with clinical information and be interpreted by registered medical practitioners only to determine final diagnosis.
7. Test results may vary based on time of collection, physiological condition of the patient, current medication or nutritional and dietary changes. Please consult your doctor or call us for any clarification.
8. Test results cannot be used for Medico legal purposes.
9. In case of queries please call customer care (91115 91115) within 48 hours of the report.

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